

primary morphology

Feature

Definition

macule

a circumscribed, flat lesion with color change up to 1 cm in diameter

patch

a flat lesion greater than 1 cm in diameter

papule

a raised lesion up to 1 cm in diameter

plaque

a raised lesion > 1 cm
in diameter

nodule

a raised lesion with
deeper vertical
dimension often
extending the
subcutaneous tissue

ulcer

full-thickness loss of the epidermis with damage into the dermis, heals with scarring

abscess

large collection of pus greater than 1cm in diameter. Often deeper and can extend into subcutaneous tissue

phlebitis

inflammation of blood
vessels

ecchymosis

bruising (does not
blanche when pressed)

pustule

an elevated usually
yellow-topped lesion
that contain pus less
than 1cm in diameter

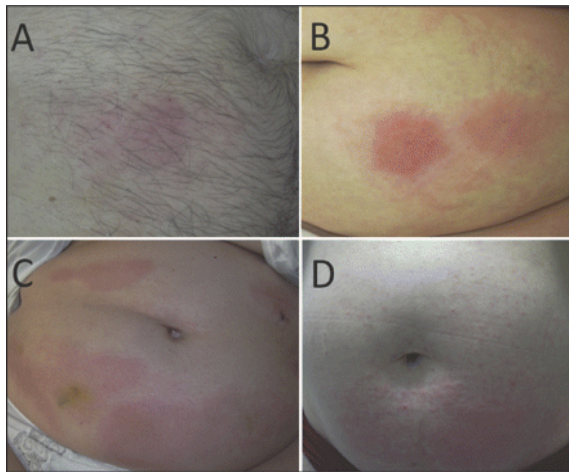
vesicle

a small, fluid-filled
blister up to 1cm in
diameter

bull

a fluid-filled blister
greater than 1 cm in
diameter

Picture 1



Picture 1

Notes

A. A few **maculae** with few papules at a heparin-injection site.
 B: Widespread erythema and red papules originating from heparin-injection sites on the
 presentation of the patient showing bilateral erythematous well-defined plaques surrounded by ecchymotic **patches** on



positive PPD skin test (Purified Protein Derivative), or Mantoux test for active TB

FIGURE 1. The patient's site of vaccination in right deltoid exhibiting a grossly necrotic ulcer with black anesthetic eschar with a surrounding large tender ill-defined indurated erythematous plaque.



Figure 1. The patient's site of vaccination in the right deltoid exhibiting a grossly *necrotic ulcer* with black eschar with surrounding large tender ill-defined *indurated erythematous plaque*.

FIGURE 1. Cutaneous nodules surrounded by erythema on the abdomen at enfuvirtide injection sites.



Figure 1. Cutaneous **nodules** surrounded by *erythema* on the abdomen at enfuvirtide injection sites.

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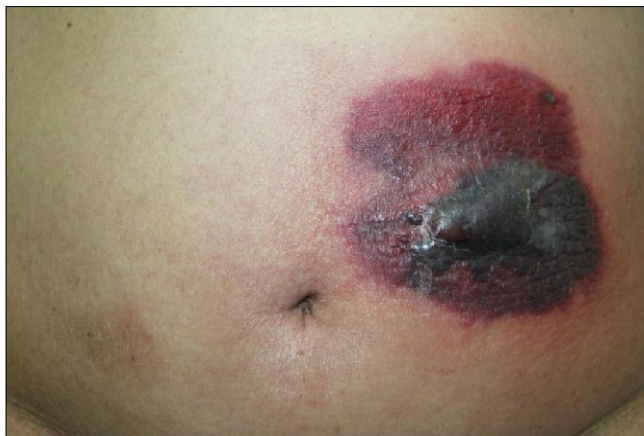
Figure 1. The patient's site of vaccination in the right deltoid exhibiting a grossly *necrotic ulcer* with black eschar with surrounding large tender ill-defined *indurated erythematous plaque*.



At this time, local examination revealed erythema, swelling and tender induration with central necrosis (figure 1). Further evaluation of the area with a CT scan revealed a well-defined area of

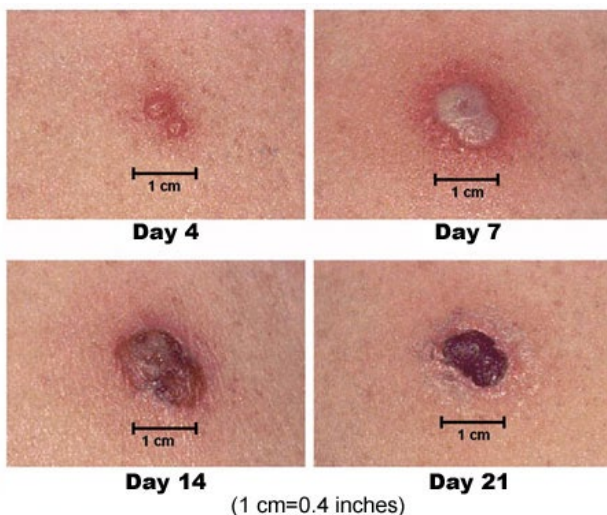


IV pethidine 25mg (diluted to 12.5 mg/ml) was given by the recovery nurse who, within five minutes, noted **severe redness in the distribution of the vein** into which it was injected **ecchymotic**



erythematous patch, with bulla is an injection-site reaction and/or Nicolau syndrome (NS), which is characterized by intense pain and ery

FIGURE 1. An ecchymotic erythematous patch with bulla to which



thematous- The pictures show a typical reaction to the smallpox vaccine. First, a small bump (papule) forms at the vaccine site 3 to 5 days after vaccination. This becomes a pus-filled



Day 2
following
inoculation
with initial
formation of
vesicles with
surrounding
erythema .



Day 5 after
inoculation,
the reaction
site with
multiple
vesicles
coalescing
into a primary
4 cm **bulla**.

Picture 2



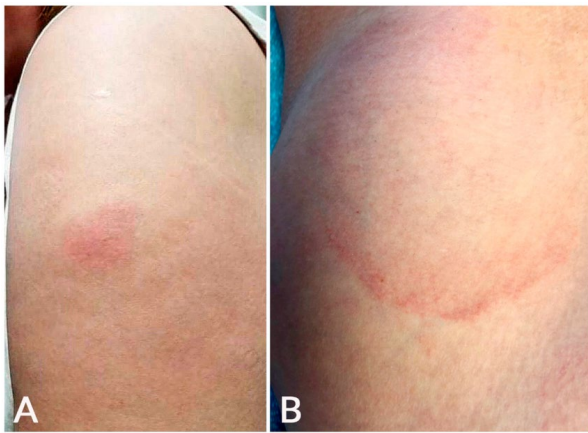
Picture 2 Notes

faint,
erythematous
macules,
which rapidly
evolved into
an
erythematous ,
indurated
plaque
surmounted by
a *bulla*

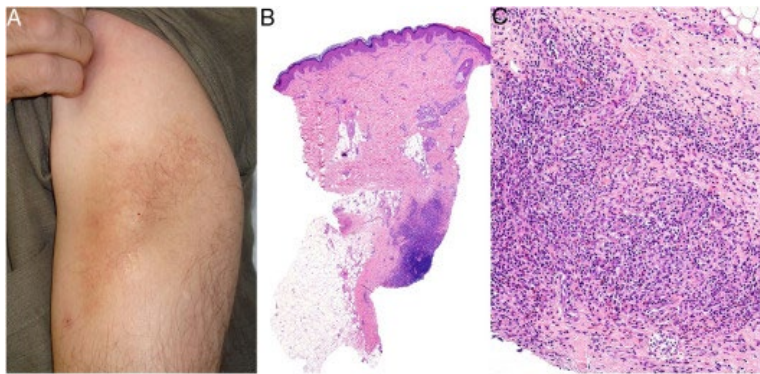
COVID-19;
annular lesion
on right



Hives (Urticaria)



Local injection
site reaction:
An
erythematous
plaque
developed at
the injection
site on the left
arm 1 day
after
BNT162b2
vaccination
(A); delayed
large, local
reaction: an
erythematous ,
annular , mildly
tender **plaque**
developed 6
days after

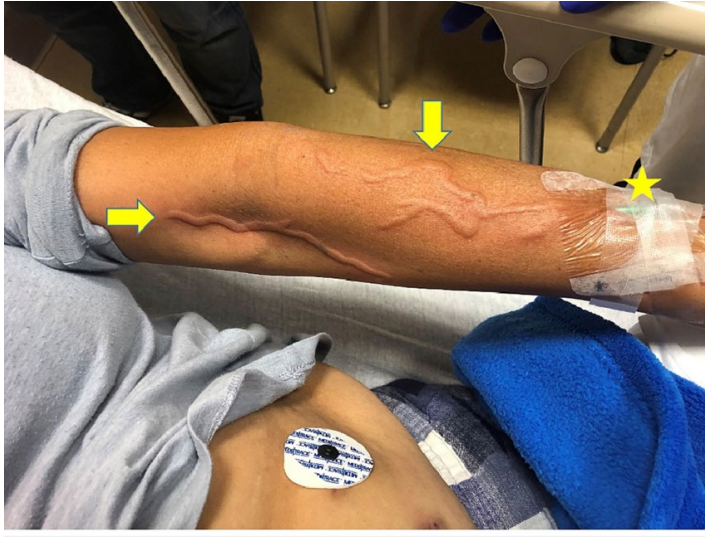


ISR - nodule



Fig 1 **Ulcer** at site of fourth injection on the upper portion of the arm.





Phlebitis

reaction on the patient's left arm (arrows) in the venous drainage of the vancomycin infusion. The infusing cannula is marked (star).



Mild **ecchymosis** occurring after HA injection at the level of the fourth intermetacarpal space



BCG Vaccine



Figure 1b. A close up view of the lesion, showing eroded vesicles with serous discharge and clustered vesicles coalescing into bullae on an erythematous base. Background photodamage is visible.



Fig. 1 An erythematous erythematous patch with bulla formation

Figure 1b. A close up view of the lesion, showing eroded **vesicles** with serous discharge and clustered vesicles coalescing into *bullae* on an erythematous base.

Background photodamage erythematous patch, with **bulla** is an injection-site reaction and/or Nicolau syndrome (NS), which is characterized by intense pain and erythematous-ecchymosis lesions at the injection- site, and can lead

Picture 3

Picture 3 Notes



**as per Dr. Sarin -
patch**



)



Erythematous
plaque at the
injection site
8 days following
the second dose
of the
Pfizer–BioNTech
vaccination. (b)
Swelling is
evident.



Fig 1 Two firm
subcutaneous
nodules with
overlying
erythema on the
right abdomen.



Fig. 1 A 4x3-cm necrotic ulcer on the right upper

Fig. 1 A 4x3-cm **necrotic ulcer** on the right upper arm covered with **eschar** at the INF- β -1b injection site and surrounded by wide, irregular **erythema**.



Severe adverse reactions in injection site were classified as 4 types. (A) Ulceration. (B) Lichenoid lesions. (C) Granulation hyperplasia. (D) **Deep abscesses**.



Phlebitis of the right hand.



erythematous, slightly indurated plaque on the anterior right thigh; slight **ecchymosis**



After tapering prednisone, the patient developed a circular plaque bordered with pustules superimposed on erythematous skin. The pustules expanded centrifugally.



Confluent,
erythematous
edematous
plaques with
vesicles at the
sites of injection
of mesotherapy
on the thigh.



faint,
*erythematous
macules* , which
rapidly evolved
into an
*erythematous ,
indurated plaque*
surmounted by a
bulla

Picture 4

Picture 4 Notes



erythematous,
slightly
indurated
plaque on the
anterior right
thigh; slight
ecchymosis

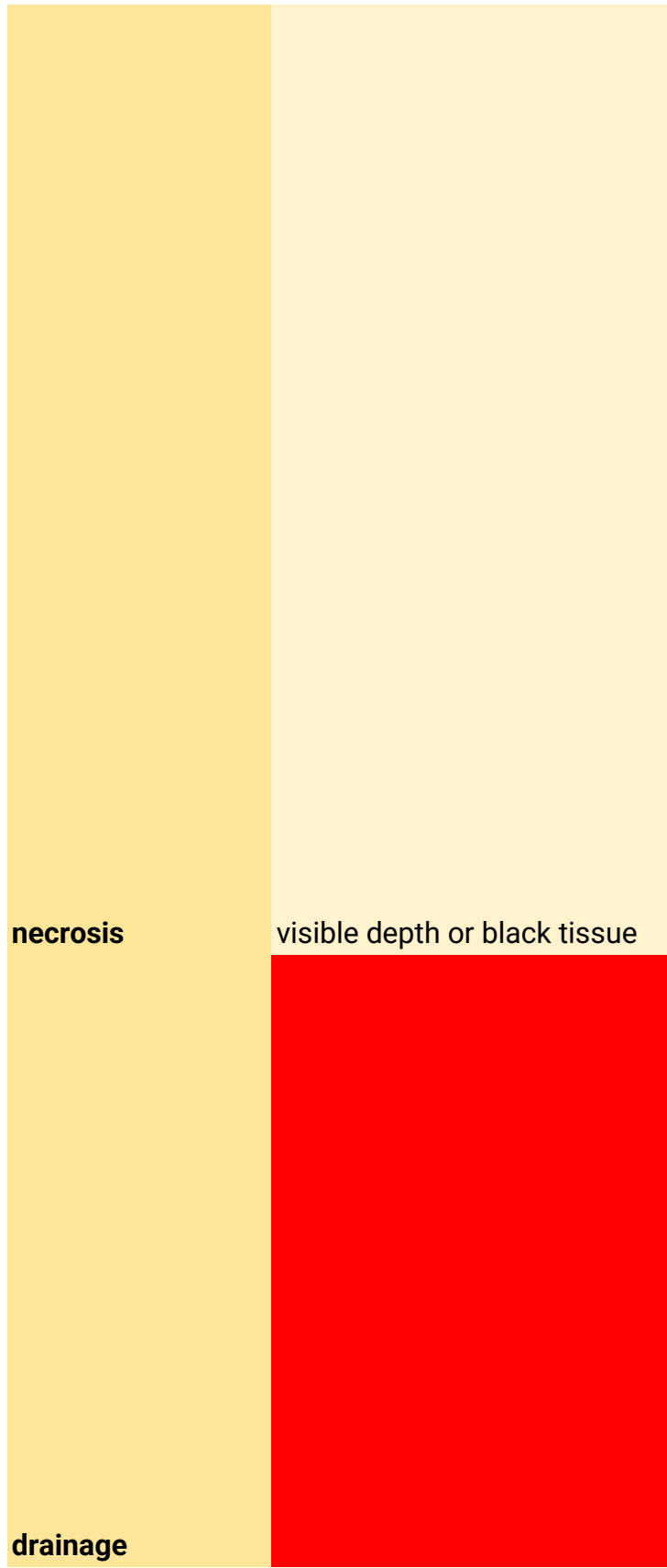


Two weeks
follow-up after
discontinuation
of liraglutide
showing
excellent
resolving of
the rash with
new deeply
seated, hard,
firm **nodules**
developed at
the same site



Severe adverse reactions in injection site were classified as 4 types. (A) **Ulceration.** (B) Lichenoid lesions. (C) Granulation hyperplasia. (D) *Deep abscesses.*

secondary morphology	
Feature	Descriptors
erythema	redness lasting >30 min after injection, blanchable
	White Red Purple Brown Yellow Black Blue
color	



edema

Fluid in tissue; soft; swelling

induration

Firmness and thickening of the intradermal space; can only be determined by *palpation* , similar to nodule

hyperpigmentation darkening of skin

hypopigmentation lightening of skin

annular

circular, ring-like ***

localized shrinking of the skin which results in paper-thin, wrinkled skin with easily visible vessels. Results from loss of epidermis, dermis or both. Dermal atrophy manifests as a depression in the skin which can occur secondary to intralesional steroid injections. Epidermal atrophy manifests as thin almost transparent skin; may not retain normal skin lines which can occur secondary to topical steroid use.

atrophy

crust

occurs from dried exudate overlying and impaired epidermis. The exudate can be composed of blood, serum, or pus. e.g impetigo, epidermolysis bullosa.

erosion

intraepithelial loss of epidermis, usually heals without scarring, moist, circumscribed, usually depressed lesion due to loss of all or part of the epidermis e.g. herpes simplex.

permanent fibrotic skin changes that develop as a consequence of tissue injury in which normal tissue is replaced by fibrous connective tissue at the site of injury to the dermis. Scars may be hypertrophic, **atrophic**, sclerotic or hard due to collagen proliferation. Reflects pattern of healing in the affected area.

scar

Picture 1



Picture 1 Notes

Day 2
following
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with initial
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close up view of the lesion, showing eroded vesicles with serous **discharge** and clustered vesicles coalescing into *bullae* on an *erythematous base*. Background photodamage



Figure 1b. A close up view of the lesion, showing eroded vesicles with serous discharge and clustered vesicles coalescing into bullae on an erythematous base. Background photodamage is visible.



Confluent, erythematous edematous plaques with vesicles at the sites of injection of mesotherapy on the thigh.

At the time, local examination revealed erythema, swelling and tender induration with central necrosis (figure 1).

Further evaluation of the area with a CT scan revealed a





Day 7 post-inoculation with newly denuded skin with circumferential
|
hyperpigmentation.



(A) Skin **hypopigmentation** and subcutaneous fat *atrophy* in the right elbow of the patient after corticosteroid injections; (B) elbow surface 24 months after fat graft.



site reaction:
An
*erythematous
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BNT162b2
vaccination
(A); delayed
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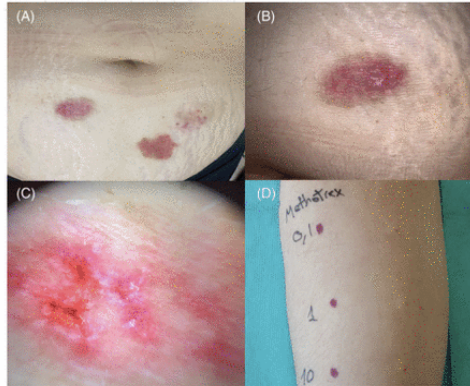
(A) Skin
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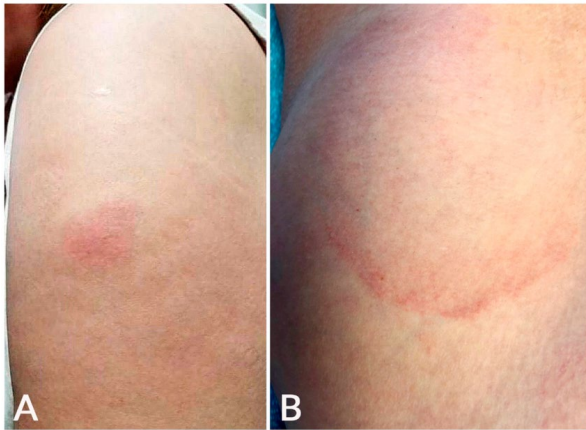
“Figure 1b. A close up view of the lesion, showing **eroded vesicles** with serous *discharge* and clustered *vesicles* coalescing into *bullae* on an *erythematous* base. Background photodamage is visible.”

FIGURE 1 (A and B) Erosive, erythematous-violaceous plaques, located in lower abdomen, around injection sites of subcutaneous MTX. (C) Dermatoscopic image: erosion and delicate scaling over the surface of a lesion. (D) IDT performed with 0.1, 1, and 10 mg/ml concentrations of MTX was negative in immediate (20 min) and late reading (7 days). IDT, intradermal test; MTX, methotrexate



Erosive,
erythematous-
violaceous
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located in
lower
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around
injection sites
of
subcutaneous
MTX.
(C)
Dermatoscopi
c image:
erosion and
delicate
scaling over
the surface of
a lesion.
(D) IDT
performed
with 0.1, 1,
and 10 mg/ml
concentration

Picture 2



Picture 2 Notes

Local injection site reaction: An **erythematous plaque** developed at the injection site on the left arm 1 day after BNT162b2 vaccination (A); delayed large, local reaction: an *erythematous, annular, mildly tender plaque* developed 6 days after mRNA-1273 vaccination at the injection site on the left arm (B).



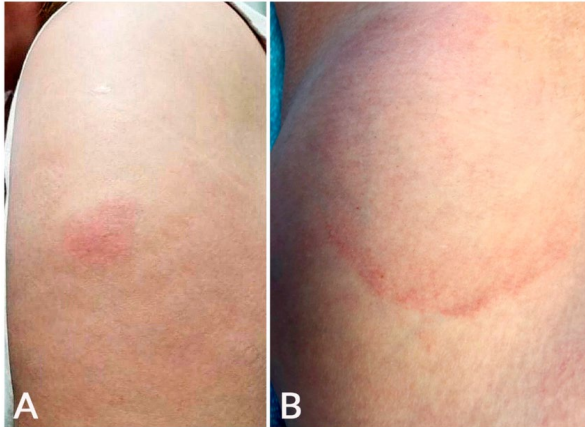
Fig. 1 A 4 × 3-cm necrotic ulcer on the right upper

Fig. 1 A 4 × 3-cm *necrotic* ulcer on the right upper arm covered with **eschar** at the INF- β -1b injection site and surrounded by wide, irregular *erythema*.



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erythema .



Erythematous plaque at the injection site 8 days following the second dose of the Pfizer–BioNTech vaccination. (b) **Swelling** is evident.



erythematous, slightly **indurated** plaque on the anterior right thigh; slight *ecchymosis*



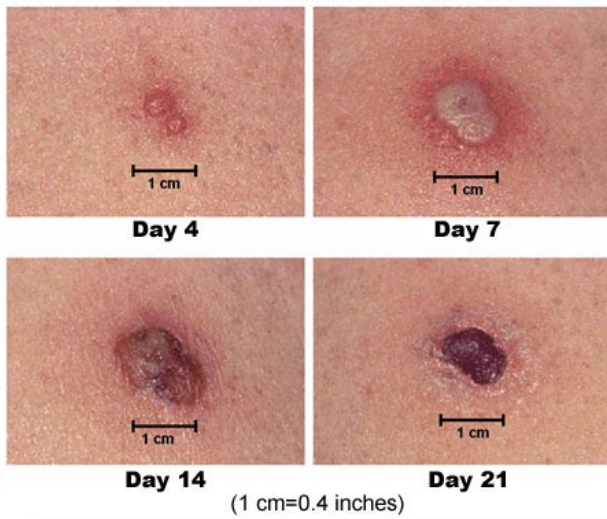
Figure 2 Marked improvement in soft tissue contour and **skin pigmentation** after 2 fat-grafting procedures.



COVID-19; annular lesion on right



Figure 1 Soft tissue **atrophy** and skin discoloration 10 months after corticosteroid injections for lateral epicondylitis.



a typical reaction to the smallpox vaccine. First, a small bump (papule) forms at the vaccine site 3 to 5 days after vaccination. This becomes a pus-filled area (*pustule*) a few days later. The pustule dries up and then forms a **scab** by about 14 days after vaccination. The scab finally falls off at about 21 days after vaccination.

Picture 3

Picture 3 Notes



Erythematous
plaque at the
injection site
8 days following
the second dose
of the
Pfizer–BioNTec
h vaccination.
(b) Swelling is
evident.



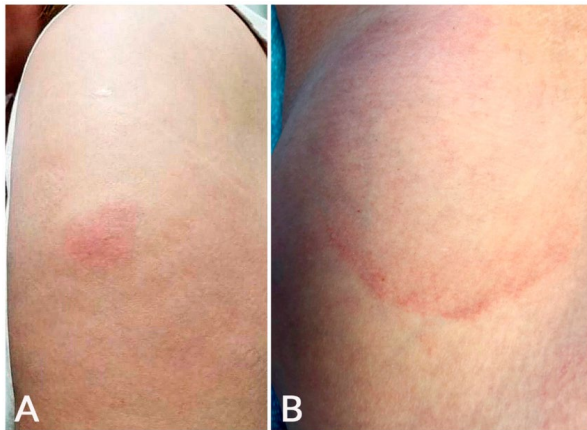
light brown
bruising



At this time, local examination revealed erythema, swelling and tender induration with central **necrosis** (figure 1). Further evaluation of the area with a CT scan revealed a well-defined area of soft tissue inflammation with central gas collection



faint,
erythematous
macules , which
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Erythematous plaque at the injection site 8 days following the second dose of the Pfizer–BioNTech vaccination. **(b)** Swelling is evident (**as per Dr. Sarin - annular lesion**)

Picture 4



Picture 4 Notes

Delayed injection-site reactions (those with onset on or after day 8) were noted in 244 participants (0.8%) after the first dose and in 68 participants (0.2%) after the second dose. Reactions were characterized by **erythema**, induration, and tenderness, and they resolved over the following 4 to 5 days."



Figure 2 Marked improvement in soft tissue contour and skin pigmentation after 2 fat-grafting procedures.

Picture 5



Picture 5
Notes

Fig 1 Two firm subcutaneous *nodules* with overlying **erythema** on the right abdomen.

Picture 6



Fig 1 A 4x3-cm necrotic ulcer on the right upper

Picture 6 Notes

Fig. 1 A 4; 3-cm
necrotic ulcer
on the right
upper arm
covered with
eschar at the
INF- β -1b
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